BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. Agree who books each check, reviews results and gives advice if a check is missed.

Check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Blood count, kidney and liver	Baseline blood count, kidney and liver profile	Review results and symptoms before treatment	Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts	Repeat sooner if unwell or results need follow-up	Follow unresolved abnormalities after treatment
Treatment review	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan
Electrolytes and EGFR effects	Check electrolyte and skin-risk plan	Skin care and prompt reaction reporting	Monitor magnesium, calcium, potassium, skin and eyes	Do not stop follow-up because the infusion stops	Monthly electrolytes, magnesium and calcium for two months after the last panitumumab dose
Heart assessment	Review cardiac history; baseline electrocardiogram (ECG) if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
international normalised ratio (blood-clotting test) (INR) only if taking warfarin	Tell the team about warfarin; arrange INR monitoring	INR as clinically indicated, rather than a universal weekly schedule	Report bleeding, bruising or significant changes in intake	The prescribing team adjusts warfarin if needed	Continue the individually agreed anticoagulation follow-up
Fluorouracil and pump safety	dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing before fluorouracil unless previously tested; confirm line access	Confirm pump education and disconnection appointment	Review mouth, bowel, cardiac and pump symptoms	Unexpected severe early effects require urgent review	Continue line care until the team removes or changes the device

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00448 v5 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Panitumumab + FOLFIRI: treatment and checks

RAS wild-type metastatic colorectal cancer; first-line or selected second-line

A 14-day cycle: panitumumab, irinotecan, folinic acid and fluorouracil start on Day 1, followed by a fluorouracil pump for about 46 hours. Arrange disconnection around Day 3. Continue according to benefit and tolerance.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Blood count, kidney and liver NCCP 00448	Baseline blood count, kidney and liver profile	Review results and symptoms before treatment	Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts	Repeat sooner if unwell or results need follow-up	Follow unresolved abnormalities after treatment
Treatment review NCCP 00448	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Electrolytes and EGFR effects NCCP 00448	Check electrolyte and skin-risk plan	Skin care and prompt reaction reporting	Monitor magnesium, calcium, potassium, skin and eyes	Do not stop follow-up because the infusion stops	Monthly electrolytes, magnesium and calcium for two months after the last panitumumab dose
Heart assessment NCCP 00448	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
INR only if taking warfarin NCCP 00448	Tell the team about warfarin; arrange INR monitoring	INR as clinically indicated, rather than a universal weekly schedule	Report bleeding, bruising or significant changes in intake	The prescribing team adjusts warfarin if needed	Continue the individually agreed anticoagulation follow-up
Fluorouracil and pump safety NCCP 00448	DPD testing before fluorouracil unless previously tested; confirm line access	Confirm pump education and disconnection appointment	Review mouth, bowel, cardiac and pump symptoms	Unexpected severe early effects require urgent review	Continue line care until the team removes or changes the device

What each check is for

1 A repeating 14-day cycle

Day ward → home pump → review

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00448

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35